



MF M

41

Support

OBSTETRICS
EXAMS Q
SUPPORT 41

End Module 41 G1

What is the living ligature of uterus?

- a) Endometrium
- b) Inner layer of myometrium
- c) Middle layer of myometrium
- d) Endo-myometrial junction
- e) Round ligament of the uterus

All of the following medications cause abortion EXCEPT:

- a) Oxytocin
- b) Prostaglandins
- c) Atosiban

Which of the following causes abortion in first trimester?

- a) Trauma
- b) Cervical incompetence
- c) Aneuploidy

According to cervical incompetence, all the following occur with it EXCEPT:

- a) Abortion occurs in first trimester
- b) Abortion occurs in second trimester
- c) Cervical cerclage is beneficial in it

Which one of the following maternal heart diseases is associated with highest mortality during pregnancy?

- a) Eisenmenger syndrome
- b) Mitral stenosis
- c) Coarctation of aorta
- d) Aortic stenosis

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Dose of Anti-D gamma globulin following first trimester abortion is:

- a) 50 microgram
- b) 100 microgram
- c) 200 microgram
- d) 300 microgram
- e) 150 microgram

All the following are causes of non-immune hydrops fetalis EXCEPT:

- a) Parvovirus infection
- b) Down's syndrome
- c) Alpha-thalassemia
- d) Rh incompatibility
- e) Twin to twin transfusion

According to Bishop score, all are true EXCEPT:

- a) Cervical dilatation
- b) Cervical effacement
- c) Cervical position
- d) Station of head
- e) Uterine contraction

When shoulder pain develops in case of tubal pregnancy, it indicates:

- a) Tubal abortion
- b) Development of tubal mole
- c) Development of broad ligament hematoma
- d) Severe internal bleeding
- e) Undisturbed ectopic pregnancy

The following conditions may be associated with molar pregnancy EXCEPT:

- a) Pregnancy-induced hypertension
- b) Gestational diabetes
- c) Thyrotoxicosis
- d) Hyperemesis gravidarum
- e) Pre-eclampsia

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To follow up after evacuation of vesicular mole:

- a) BhCG level
- b) Ultrasound
- c) Progesterone

Vesicular mole may be associated with:

- a) Theca lutein cysts
- b) Follicular ovarian cysts
- c) Ovarian carcinoma
- d) Ovarian atrophy
- e) Ovarian thecoma

The smallest fetal skull diameter in the following:

- a) Submento-bregmatic
- b) Suboccipito-frontal
- c) Mento-vertical
- d) Submento-vertical

Which of the following tests is the most sensitive for detection of iron depletion in pregnancy:

- a) Serum iron
- b) Serum transferrin
- c) Serum ferritin
- d) Serum erythropoietin
- e) Iron binding capacity

Which of the following organisms causes acute pyelonephritis:

- a) E. coli
- b) Chlamydia
- c) Toxoplasmosis

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Trial of labor can be done in:

- a) Mild cephalopelvic disproportion
- b) Severe cephalopelvic disproportion
- c) Breech presentation
- d) Shoulder presentation
- e) Face presentation

Drugs used for tocolysis include the following, EXCEPT:

- a) Indomethacin
- b) Nifedipine
- c) Atosiban
- d) Magnesium sulfate
- e) Hydralazine

A woman was in delivery and after it there is something protruding from the vagina:

- a) Acute uterine inversion
- b) Uterine prolapse
- c) Uterine fibroid

Partogram is used to record:

- a) ^{Progress} ~~Extent~~ of labour
- b) Fetal assessment
- c) Uterine position
- d) Cervical incontinence

With which of the following types of viral hepatitis in pregnancy, the maternal mortality is the highest?

- a) Hepatitis A
- b) Hepatitis B
- c) Hepatitis C
- d) Hepatitis E
- e) Hepatitis D

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Which of the following can't be delivered from vagina:

- a) Previous C.S.
- b) Placenta previa centralis
- c) Breech presentation
- d) Dead fetus

The one measurement of fetal maturity that is not affected by a "bloody tap" during amniocentesis is:

- a) L/S ratio
- b) Phosphatidylglycerol
- c) Alpha fetoprotein
- d) Bilirubin as measured by DDD 450
- e) Fetal fibronectin

Face to pubis delivery is possible with which cephalic presentation:

- a) Mento-anterior
- b) Mento-posterior
- c) Occipito-posterior
- d) Brow presentation
- e) Occipito-anterior

Most important symptom of undisturbed ectopic pregnancy:

- a) Amenorrhea
- b) Abdominal pain
- c) Vaginal bleeding
- d) Shoulder pain

Appropriate material for antenatal diagnosis of genetic disorders include all of the following EXCEPT:

- a) Fetal blood
- b) Amniotic fluid
- c) Chorionic villi
- d) Maternal urine

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Non-contraceptive advantage for combined oral contraceptives (COCs):

- a) Increase iron deficiency anemia
- b) Increase incidence of endometrial cancer
- c) Protect against ovarian cysts

Milk engorgement management:

- a) Warm towel and continue lactation

First antenatal care visit:

- a) As early as possible

• سؤالين عن الـ Red flags

الأول إجابته Vaginal bleeding during pregnancy
الثاني إجابته Post partum والضغط أقل من 90/60

End Module 40 G1

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- c) Chorionic villi
- d) Maternal urine

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Which of the following is not considered a risk factor for high-risk pregnancy?

- a) Multiple pregnancy
- b) Elevated serum cholesterol level
- c) History of domestic violence
- d) Previous history of macrosomic baby
- e) Intrauterine growth retardation

A 22-year-old primigravida delivered a healthy male infant 10 days ago by cesarean section. Which of the following symptoms requires urgent referral?

- a) Low back pain
- b) Nipple fissures or cracking
- c) Dyspnea
- d) Abdominal distension and flatulence
- e) Fatigue and lack of sleep

You are counselling a 26-year-old female about contraceptive methods. Which of the following is true regarding combined vaginal rings?

- a) The ring is inserted in the vagina 2 hours before sexual intercourse
- b) It is contraindicated in obese females
- c) It acts locally by killing sperms and changing cervical mucous
- d) The ring is left in place for 3 weeks, then removed for the fourth week
- e) Combined vaginal ring is not preferred for young females

When counselling a pregnant woman about healthy diet, which of the following is included in the second layer of the food guide pyramid?

- a) Vegetables
- b) Dairy products
- c) Meats
- d) Grains
- e) Fats, oils, and sweets

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All of the following conditions may be improved with the use of combined hormonal contraceptives EXCEPT:

- a) Iron-deficiency anemia
- b) Ectopic pregnancy
- c) Dysmenorrhea
- d) Acne
- e) Cholelithiasis

A pregnant lady in the 10th week of gestation. Which of the following is not included in the educational message for her?

- a) Nutritional counselling
- b) Personal hygiene and breast care
- c) Exercise and proper daily activity
- d) Promotion for breastfeeding
- e) Dangerous signs during pregnancy

A 22-week pregnant woman needs to know if there is an error or issue in the fetus by ultrasound. It should be done every:

- a) 10–14 days
- b) 4–7 days
- c) 21 days

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A woman with 20 weeks gestation presents with vaginal bleeding. On speculum examination, the cervical os is open but no products have come out. The diagnosis is:

- a) Inevitable abortion
- b) Missed abortion
- c) Complete abortion
- d) Incomplete abortion
- e) Tubal abortion

All the following are components of Biophysical Profile EXCEPT:

- a) Non stress test
- b) Fetal body movement
- c) Fetal respiratory movement
- d) Oxytocin challenge test
- e) Amniotic fluid volume

Eclampsia is differentiated from preeclampsia by:

- a) Proteinuria
- b) Convulsions
- c) Papilledema
- d) Retinal hemorrhage
- e) Lower limb edema

The dose of anti-D immunoglobulin given after term delivery for a Rh-negative mother with Rh-positive baby is:

- a) 50
- b) 200
- c) 300
- d) 100
- e) 150

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What is the engaging diameter in brow presentation?

- a) Mento-vertical diameter
- b) Sub-mento-bregmatic diameter
- c) Sub-mento-vertical diameter
- d) Occipito-vertical diameter
- e) Occipito-frontal diameter

Caudal regression syndrome is seen in babies of mother having:

- a) Diabetes mellitus
- b) Pregnancy induced hypertension
- c) Cardiac disease
- d) Rh incompatibility
- e) Systemic lupus erythematosus

All of the following are normal maternal physiological changes during pregnancy EXCEPT:

- a) Increased cardiac output
- b) Decreased fibrinogen
- c) Increased tidal volume
- d) Constipation
- e) Increased glomerular filtration rate

What is true about obstetric examination for complete breech presentation?

- a) Fundal level less than period of amenorrhea
- b) In fundal grip head felt with difficulty
- c) In umbilical grip, limbs are felt on either side of middle line
- d) In pelvic grip, buttocks not engaged
- e) FHS are heard at umbilicus

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A pregnant woman is found to have excessive accumulation of amniotic fluid. Such polyhydramnios is likely to be associated with the following conditions EXCEPT:

- a) Twinning
- b) Anencephaly
- c) Esophageal atresia
- d) Diabetes mellitus
- e) Bilateral renal agenesis

A 22-week primigravida had her first day of LMP on June 10. She had regular 32-day cycles prior to that pregnancy. Her expected date of delivery would be:

- a) March 17
- b) March 22
- c) March 13
- d) March 21
- e) March 10

A drop in the fetal heart rate that typically lasts less than two minutes and usually associated with umbilical cord compression is called:

- a) Early deceleration
- b) Variable deceleration
- c) Late deceleration
- d) Prolonged deceleration
- e) Transient acceleration

Treatment of theca lutein ovarian cysts that are found in association with hydatidiform mole is:

- a) Ovarian cystectomy
- b) Ovariectomy
- c) Methotrexate
- d) Ovariotomy
- e) Suction evacuation

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The screening test for gestational diabetes mellitus that has the highest sensitivity is:

- a) Glycosylated Hb
- b) Fasting blood sugar
- c) 50-gram glucose challenge test
- d) Random blood sugar
- e) Glucose in urine

Your patient has microcytic anemia with a hemoglobin of 9 and normal serum ferritin. What is the most likely diagnosis?

- a) Folate deficiency
- b) Vitamin B12 deficiency
- c) Thalassemia
- d) Vitamin B6 deficiency
- e) Megaloblastic anemia

Leopold maneuver refers to method for:

- a) Delivery of after-coming head of breech
- b) External cephalic version
- c) Internal podalic version
- d) Breech extraction
- e) Obstetric examination of abdomen

The best method for shortening the second stage of labor in patients with heart disease is by:

- a) Prophylactic forceps
- b) Prophylactic Ventouse
- c) Upper segment cesarean section
- d) Spontaneous delivery with episiotomy
- e) Lower segment cesarean section

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Ligation of which artery can control postpartum hemorrhage:

- a) Origin of internal iliac artery
- b) Anterior division of internal iliac artery
- c) Posterior division of internal iliac artery
- d) Common iliac artery
- e) External iliac artery

The smallest anteroposterior diameter of the pelvic inlet is:

- a) Interspinous diameter
- b) True conjugate diameter
- c) Diagonal conjugate
- d) Obstetric conjugate
- e) Bi-ischial

The relation of the fetal parts to each other determines which of the following?

- a) Presentation of the fetus
- b) Attitude of the fetus
- c) Lie of the fetus
- d) Position of the fetus
- e) Denominator of the fetus

The most potent drug to control atonic postpartum hemorrhage amongst the options:

- a) Oxytocin
- b) Misoprostol
- c) Dinoprostone
- d) Methotrexate
- e) Carboprost

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What is the mechanism by which occiput rotates 1/8 of circle outside vulva?

- a) Restitution
- b) Extension
- c) Deflexion
- d) Expulsion
- e) Flexion

Folic acid supplement reduces the risk of:

- a) Anencephaly
- b) Preeclampsia
- c) Down syndrome
- d) Placenta previa
- e) Congenital heart disease

Which of the following is the indication of internal podalic version:

- a) 2nd twin
- b) Shoulder presentation
- c) Breech presentation
- d) Mento-posterior
- e) Occipito-posterior

Preterm rupture of the membranes is most strictly defined as spontaneous rupture at any time prior to which of the following?

- a) A stage of fetal viability
- b) The second stage of labor
- c) The 32nd week of gestation
- d) The onset of labor
- e) The 37th week of gestation

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How to diagnose false labor pains?

- a) Regular
- b) Voluntary
- c) Relieved by enema
- d) Associated with dilation of cervix
- e) Increase in frequency or duration

During the first ANC visit for 10 weeks pregnant female. You are convincing her to have her first ultrasound testing. Which of the following isn't true regarding U/S for this lady?

- a) It is the best time to assess EDD
- b) Can estimate gestational age
- c) Assess fetal heart beats
- d) Detect the multiple pregnancy
- e) Can detect fetal congenital anomalies early

A 19-year-old woman is seen in the office 3 weeks postpartum. She is exclusively breastfeeding and has not had a menstrual cycle since her delivery. She would like to have an IUD placed for contraception, as she would like to wait several years before having another baby. Which of the following actions would be most appropriate at this time?

- a) Plan to insert the IUD at a 6-week postpartum visit
- b) Prescribe progestin-only minipills until she is no longer breastfeeding and then insert the IUD
- c) Advise that she needs no contraception until she is no longer breastfeeding and she should return after that time for the IUD
- d) Insert the IUD today

When counseling a pregnant lady about getting enough iron in her diet, you should tell her which of the following?

- a) Milk, coffee, and tea aid iron absorption if consumed at the same time as iron
- b) Iron absorption is inhibited by a diet rich in vitamin C
- c) Iron supplements are permissible for children in small doses
- d) Constipation is common with iron supplements

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A 22-year-old pregnant woman has hemoglobin of 11.5 g/dL at eight weeks of gestation, which of the following is true?

- a) She carries an increased risk of delivering a low-birth-weight baby
- b) High protein food supplements are indicated
- c) It is best to start taking oral iron supplements every day as soon as possible
- d) Weekly oral iron supplements should be taken as soon as possible
- e) Counseling about healthy eating and keeping physically active during pregnancy is recommended

Which of the following blood tests would you NOT routinely request for 21-year-old lady at eight weeks of her pregnancy?

- a) CBC
- b) Blood type
- c) Rh
- d) Oral glucose tolerance

A 23-year-old female with regular menstruation comes to the clinic seeking IUD insertion. Which of the following is the optimal period for inserting an IUD?

- a) The IUD can be inserted at any time throughout the menstrual cycle
- b) On the first day of menstruation
- c) Few days before the expected time of menstruation
- d) Between days 4 and 6 of the menses
- e) About 4-6 day after caseation of menstruation

A 35-year-old pregnant female who has two children. She plans to get pregnant again. She asked her physician if she needs to repeat the genetic testing before pregnancy. Which of the following is not an indication for genetic testing in this female?

- a) There is history of recurrent abortion
- b) History of previous child with thalassemia
- c) Congenital abnormalities in the offspring
- d) Previous history of stillbirth
- e) Consanguinity between parents

Final 40

- Define recurrent abortion, Enumerate 3 causes, List lines of treatment
- Define hyperemesis gravidarum, Mention 2 theories for pathogenesis, Mention 2 complications, List lines of treatment
- Define intrauterine growth restriction, Enumerate 2 risk factors, Mention 2 complications, List 3 investigations with findings in IUGR
- Define retained placenta, Mention 2 causes of non-separated retained placenta, Mention line of management
- A 30-year-old patient, G4/P4+1, presents to the antenatal clinic complaining of minimal vaginal bleeding. She reports that her last menstrual period was one month ago. She has a history of IUCD insertion 3 years ago and now complains of offensive vaginal discharge. On examination, her BP, pulse, and temperature are normal, but she has a tender right adnexa on vaginal examination. Pregnancy test is positive. Ultrasound shows empty uterus, IUCD in place, and a 2 cm right adnexal mass.

1. What is the most probable diagnosis?

2. What is your next step to confirm your diagnosis?

3. Outline 3 indications for expectant management of this condition

4. List 3 contraindications for medical treatment of this condition

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- A 26-year-old woman, pregnant at 32 weeks, comes to the antenatal clinic complaining of watery vaginal discharge that increases with coughing and straining. Her vital signs are within normal limits. Ultrasound shows a viable fetus. The last ultrasound done 3 weeks ago showed an AFI of 25.

1. What is your provisional diagnosis?

2. What is the predisposing factor for this condition in this case?

3. What is the first step to confirm the diagnosis?

4. List 5 lines of conservative management.

- A 35-year-old patient, G6P5+1, pregnant at 37 weeks with twin pregnancy (both cephalic), came to the emergency room complaining of labor pain since the day before. On examination, BP, pulse, and temperature were normal. Ultrasound showed both fetuses were alive and in cephalic presentation. On PV examination, cervix was fully dilated. She delivered immediately, but after delivery of the placenta, severe vaginal bleeding occurred. Her uterus was relaxed and blood clots were coming from the vagina

1. What is your most probable diagnosis?

2. Mention the risk factor that led to this condition in this case.

3. How can you prevent this condition?

4. Enumerate steps of management of this condition.

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- Ms. Nadia, 20 years old, primigravida at 29 weeks, came to the antenatal clinic complaining of tachycardia, fatigue, shortness of breath, and pallor. Her BP and temperature were normal. Her hemoglobin was 8.5 g/dL. She received iron infusion, but her Hb level did not improve

1. List the investigation of choice to confirm your diagnosis.

2. Enumerate 2 factors that inhibit iron absorption.

3. Mention 2 contraindications for intravenous iron.

4. When is the patient referred for blood transfusion?

- Match each of the following clinical presentations with the most appropriate line of management

1. Patient with placenta previa and severe bleeding with a preterm fetus. **D**

2. Patient with central placenta previa at 37 weeks gestation. **C**

3. Patient with placenta accreta and life-threatening hemorrhage. **E**

4. Patient with mild placental abruption, no fetal distress, at 33 weeks. **A**

5. Patient with severe accidental hemorrhage, IUFD, and 8 cm dilated cervix. **B**

Options:

- a) Conservative treatment
- b) Allowance or trial of vaginal delivery
- c) Elective cesarean section
- d) Emergency cesarean section
- e) Cesarean hysterectomy
- f) Tocolysis

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Regarding twin-to-twin transfusion syndrome (TTTS), which of the following statements is false?

- a) Unidirectional anastomoses are more likely
- b) It is more common in monochorionic diamniotic (MCDA) pregnancies
- c) Presence of twin peak sign on ultrasonography suggests dichorionic placenta
- d) Fetal survival is better in artery-vein anastomosis
- e) Can cause fetal hydrops

Regarding puerperal pyrexia, the following statements are true EXCEPT:

- a) Maternal diabetes is a high risk factor
- b) Group A β -haemolytic streptococcus is a known pathogen
- c) Patient may present with features of pneumonia
- d) Infection is monomicrobial in majority
- e) Reactivation of pulmonary tuberculosis may be a cause

Regarding hyperemesis gravidarum, all are correct EXCEPT:

- a) It is associated with weight loss of more than 5% of the body mass
- b) Glycosuria is commonly observed
- c) Thiamine supplement is helpful
- d) Omeprazole may relieve the symptoms
- e) Common in twin pregnancy

The following are predisposing factors for placenta previa, EXCEPT:

- a) Advancing age
- b) High order pregnancy
- c) Previous caesarean section
- d) Malpresentation
- e) D&C

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The physiological iron deficiency state during pregnancy is evidenced by the following EXCEPT:

- a) Fall in haemoglobin concentration
- b) Fall in haematocrit
- c) Low serum iron
- d) Low iron binding capacity
- e) Low serum ferritin

The following are related to the treatment of thalassemia EXCEPT:

- a) Fresh blood transfusion
- b) Folic acid
- c) Routine iron therapy
- d) Deferoxamine improves pregnancy outcome
- e) Withhold iron therapy

Patients with organic heart disease in pregnancy most commonly die during:

- a) 20–24 weeks of pregnancy
- b) First stage of labour
- c) Soon following delivery
- d) Two weeks postpartum
- e) First trimester of pregnancy

The best method of shortening the second stage of labour in heart disease is by:

- a) Prophylactic forceps
- b) Ventouse
- c) Spontaneous delivery
- d) Caesarean section
- e) Episiotomy

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The following statements are related to cord prolapse EXCEPT:

- a) Common in malpresentation
- b) A loop of cord may be brought down to confirm the pulsation
- c) One of the first aid management is to lift the presenting part of the cord
- d) High risk of fetal distress
- e) Treated by emergency CS

Shoulder dystocia may occur in all EXCEPT:

- a) Big baby
- b) Anencephaly
- c) IUGR
- d) Normal size baby
- e) Of diabetic mother

A 33-year-old pregnant lady at 38 weeks gestation. CTG monitoring revealed transient decrease in fetal heart rate after beginning of uterine contractions and subsided after ending of the contractions.

What is your diagnosis?

- a) Early deceleration
- b) Late deceleration
- c) Variable deceleration
- d) Sustained deceleration
- e) Bradycardia

Oxytocin infusion is contraindicated in all EXCEPT:

- a) Obstructed labour
- b) Trial of labour
- c) Hypovolemic shock
- d) Cardiac disease
- e) Contracted pelvis

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Ms. Nora just had a positive pregnancy test and wonders if you can tell her when she is likely due. Her LMP was June 30. Her expected date of confinement (EDC) is approximately which of the following?

- a) March 23
- b) March 7
- c) March 28
- d) March 23
- e) April 7

There is good evidence that a woman who gave birth to an infant with a neural tube defect (NTD) can substantially reduce the risk of recurrence by taking periconceptional folic acid supplementation.

What is the recommended dose?

- a) 0.4 mg
- b) 0.8 mg
- c) 1.0 mg
- d) 4 mg
- e) 8 mg

Pregnancy should be avoided within 1 month of receiving which of the following vaccinations?

- a) Measles, mumps, rubella (MMR)
- b) Influenza
- c) Hepatitis B
- d) Tetanus
- e) Pneumococcus

Your patient has microcytic anemia with a hemoglobin of 9 and normal iron stores. What is the most likely diagnosis?

- a) Folate deficiency
- b) Vitamin B12 deficiency
- c) Thalassemia
- d) Vitamin B6 deficiency
- e) Acute blood loss

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Which of the following is normally associated with meconium-stained amniotic fluid?

- a) Fetal asphyxia
- b) Breech presentation
- c) Before fetal death
- d) Fetal acidosis
- e) Fetal distress

A 25-year-old pregnant lady at 39 weeks gestation. During labor, a hand is found beside the head in cephalic presentation.

What is your diagnosis?

- a) Occipito-anterior position
- b) Occipito-posterior position
- c) Face presentation
- d) Brow presentation
- e) Compound presentation

What is the station when the fetal head is at the level of ischial spines

- a) -2
- b) +1
- c) +2
- d) 0
- e) -1

Vaginal examination is contraindicated in which situation during pregnancy?

- a) Carcinoma of the cervix
- b) Gonorrhea
- c) Prolapsed cord
- d) Placenta previa
- e) Active labor with ruptured membranes more than 6 hours

SUPPORT 41

During the first ANC visit for 26 weeks pregnant female with a previous history of irregular menstruation. Which of the following isn't true regarding ultrasound for this lady?

- a) Estimation of EDD accurately
- b) Check the position of the placenta
- c) Can detect fetal congenital anomalies
- d) Detect multiple pregnancy
- e) Assess fetal heart beats

You are caring for a female on the 7th day of the postpartum period after vaginal delivery. Which of the following is proper service to provide to this lady?

- a) Advise her to use vaginal douches to prevent infection
- b) Urine analysis is an integral part in postpartum care
- c) Encourage her to breastfeed her baby exclusively
- d) Advise her to start using any contraceptive method after 6 months to avoid reduction in breast milk secretion
- e) Advise her to regain her usual physical activity after the end of puerperium

You are counselling a young married female about contraceptive methods, which of the following is true as regard progesterone releasing vaginal rings?

- a) The ring is inserted in the vagina and left in place for up to 4 weeks maximum
- b) Start each new ring immediately after removal of the previous ring
- c) It is contraindicated in obese females
- d) The ring is not suitable for lactating females
- e) Shouldn't be used for more than successive three years to avoid local gynecological cancers

SUPPORT 41

A 22-year-old lady is planning to conceive. The patient is 165 cm tall and weighs 55 kg with body mass index 20. The patient is concerned about weight gain during pregnancy.

What is the best response to her?

- a) Gaining up to 20 kg is recommended for her to avoid intrauterine growth retardation
- b) A kilogram a week throughout pregnancy is recommended
- c) The adequate weight gain is 3.5 kilograms in the first 10 weeks, then 0.5 kg per week after that
- d) The recommended gain is at the rate of approximately 0.4 kg per week in the first and second trimesters of pregnancy
- e) Gaining 12.5–18 kg is recommended for her for healthy fetal growth

Which of the following statements is true regarding the use of any combined hormonal contraceptive method?

- a) Combined hormonal contraception is contraindicated in a female who is a smoker less than 35 years
- b) Combined hormonal contraception may increase the risk of endometrial cancer
- c) Combined hormonal contraception is contraindicated if there is a history of sexually transmitted diseases
- d) Combined hormonal contraception is contraindicated in patients taking carbamazepine
- e) Combined hormonal contraception can't be used in overweight female

SUPPORT 41

- A 25-year-old pregnant female (G2 P1+0) presents for the second prenatal visit. She is at 16 weeks' gestation based on her LMP. She has no specific complaint. On doing routine investigations for her, CBC showed:

Hb: 9 g/dL

MCV: 70 micrometers (N: 75–100)

MCH: 23 pg

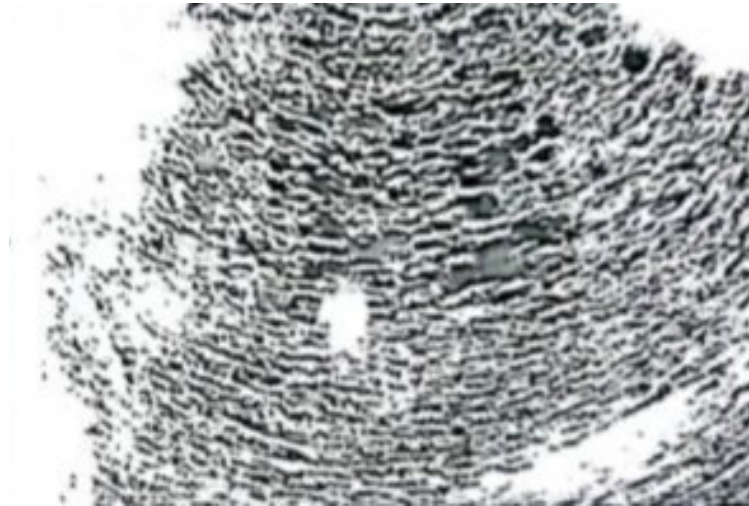
1. What is the degree of anemia this patient suffers from?
 2. What is the expected response after receiving the treatment?
 3. List three indications for giving IV iron during pregnancy.
 4. List three indications for referral regarding this medical problem.
- A 28-year-old female patient presents to the clinic with a concern regarding her intrauterine device (IUD). She reports that she cannot feel the strings of her IUD, which she has been using for contraception for the past two years. She last checked her IUD strings about a month ago and felt them normally.
1. What are the potential reasons for the missing IUD strings?
 2. List important questions during history taking that would help you to assess this condition.
 3. What are the next steps for managing this patient?

Final 39

- Enumerate 2 misleading signs in diagnosing heart disease with pregnancy, mention 2 possible fetal complication of this condition, and outline management of this condition during second stage of labor.
- Define septic abortion, enumerate 2 clinical features, and outline 2 possible surgical lines of treatment of this serious condition.
- Mention 4 advantages of ventouse over forceps application, enumerate 2 contraindications, and list 2 possible fetal complications of ventouse.
- Enumerate the denominator and positions of breech presentation, mention 2 methods for delivery of the after coming head, enumerate 2 maternal complications, and list indications for cesarean section of this condition.

SUPPORT 41

- A 26 years old multi-gravida about 7 weeks gestation was admitted to hospital suffering from mild vaginal bleeding and suprapubic pain. The patient had suffered from nausea and vomiting since 5 days, on examination, pulse was 90-min temperature 37c, BP 130-80 MMHG uterus was oversized and doughy in consistency, cervix was closed and soft serum B-BCG was 100000 mIU-m, ultrasound revealed the following image of the uterus with multiple ovarian cysts.



1. What is the most probable diagnosis?
2. Enumerate the surgical treatment of choice for this patient.
3. How can you do post treatment follow up for this patient?
4. Enumerate two indications for the chemotherapy for this patient after treatment.

SUPPORT 41

- Define intrauterine growth restriction, enumerate its types, mention 2 values of ultrasonography in this condition, and list 2 factors that determines the optimal time of delivery.
- A 34 years-old G2 with previous one cesarean section. She was about 34 weeks gestation and admitted to hospital suffering from painless recurrent mild attacks of bright red vaginal bleeding. On examination, pulse was 95/min, temperature 37.6, BP 100-70. On abdominal examination, there was a relaxed uterus and FHR 150- min by ultrasound, the placenta was completely covering the cervix.

1. What is your provisional diagnosis of this condition?

2. Enumerate 2 common factors that predispose to this condition.

3. How can you manage this condition according to the amount of vaginal bleeding and gestational age?

- A 29-year-old primigravida about 32 weeks gestation was admitted to hospital suffering from intermittent colicky lower abdominal pain and increased vaginal discharge. On examination, pulse was 90-min, temperature 37.6, BP 110-70 mmhg Active uterine contractions were recorded by CTG with 2 contractions every 10 minutes. On vaginal examination, cervical dilatation was 2 cm with 50% effacement.

1. What is the most probable diagnosis?

2. Enumerate two methods to enhance fetal lung maturity in this condition.

3. Enumerate 2 commonly used drugs for cessation of uterine contractions in this condition.

4. Enumerate 2 fetal contraindication in this condition.

SUPPORT 41

- A 39 years-old G2 with previous one cesarean section. She was about 34 weeks gestation and admitted to hospital suffering from severe headache and blurring of vision, on examination, pulse was 100-min, temperature 37c, BP 180-120 and lower limb edema. During examination, she suddenly suffered from fits urine revealed albuminuria+3, Hb 10 gm/dl, platelets 120000 cells/cc, normal liver transaminases, vaginal examination revealed closed cervix and not effaced.

1. What is your provisional diagnosis of this condition?

2. Enumerate 2 emergency first aid measures for this patient.

3. Enumerate the drug of choice for this condition and its follow up parameters?

4. Enumerate 2 maternal complications of this condition.

- A 30 year old G4 para 340 with pregestational diabetes. She was about 40 weeks gestation and admitted to hospital suffering from true labor pains. On examination, pulse was 90-min, temperature 37c, BP 110-70. Vaginal examination revealed fully dilated cervix and cephalic presentation. During delivery of the fetus, the shoulders suddenly retracted and the head snapped back against the perineum.

1. What is the most probable diagnosis and name this characteristic sign?

2. Enumerate two possible fetal complication of this condition.

3. Enumerate the sequential management steps of this serious condition.

SUPPORT 41

- For each question, select the one lettered option that is most closely associated with each lettered option may be used once, multiple times, or not at all

Options:

- A. Station
- B. Presentation
- C. Lie
- D. Attitude
- E. Anterior asynclitism
- F. Molding
- G. Posterior asynclitism
- H. Effacement

Questions:

- The cervical canal becomes incorporated into the lower uterine segment
- The fetal bones overlap each other. ^IG
- The relation of the denominator to the right or the left side of maternal pelvis C
- The first part of the fetus, which meets the pelvic brim and first felt vaginal examination. B
- The relation of fetal parts to each other. E
- The relation between the longitudinal axes of the fetus to that of the mother D

Options:

- A. Shoulder presentation
- B. Brow
- C. Complete breech
- D. Compound presentation
- E. Shoulder dystocia
- F. Frank breech
- G. Face
- H. Unstable lie

Questions:

1. Cephalic presentation in which a limb is found beside the presenting part. **D**
2. Flexion of hip joints and extension of knee joints with one or both feet found beside the head. **F**
3. There is no mechanism of labor because the head descends by mento-vertical diameter, which is longer than any pelvic diameter. **B**
4. The bony landmarks are felt during vaginal examination are the orbital margin, the nose, mouth and chin. **G**

SUPPORT 41

A 23 years old G2 P1+0 previous one cesarean section is admitted to hospital with history of dribbling for 24 hours at a period of amenorrhea of 32 weeks. Speculum examination confirms dribbling. The liquor is offensive. Her temperature is 38c. High reactive protein. No uterine contraction. FHR is 170 per minute.

What is the most appropriate treatment?

- a) administer broad spectrum antibiotics and corticosteroids and manage conservatively if the temperature decreases
- b) perform cesarean section immediately after administering 1 dose of corticosteroids and broad spectrum antibiotics
- c) augment labor with intravenous infusion of oxytocin
- d) administer tocolytics therapy
- e) repeat reactive protein 24 hours after commencing broad spectrum

The placenta of twins can be:

- a) dichorionic and monoamniotic in dizygotic twins
- b) dichorionic and monoamniotic in monozygotic twins
- c) monochorionic and monoamniotic in dizygotic twins
- d) monochorionic and diamniotic in monozygotic twins
- e) dichorionic and diamniotic in monozygotic twins

All of the following are maternal changes during pregnancy EXCEPT:

- a) increase cardiac output
- b) increase alveolar residual volume
- c) increase fibrinogen
- d) increase glomerular filtration rate
- e) increase stroke volume

Your patient has microcytic anemia with a hemoglobin of 9 and normal iron stores. What is the most likely diagnosis?

- a) folate deficiency
- b) vitamin B12 deficiency
- c) thalassemia
- d) vitamin B6 deficiency
- e) iron deficiency anemia

SUPPORT 41

A primigravida presents in emergency room with painless bleeding at 12 weeks of gestation. Cervix is closed and normal.

What is the most probable diagnosis?

- a) septic abortion
- b) placenta previa
- c) incomplete abortion
- d) inevitable abortion
- e) threatened abortion

All predispose to isoimmunization in Rh negative female EXCEPT:

- a) advanced maternal age
- b) antepartum hemorrhage
- c) cesarean section
- d) ectopic pregnancy
- e) vaginal delivery

All of the following are components of biophysical profile EXCEPT:

- a) Oxytocin challenge test
- b) Non stress test
- c) Amniotic fluid volume
- d) Fetal respiratory movement
- e) Fetal body movement

Ritgen maneuver is done in:

- a) Shoulder dystocia
- b) For delivery of the head in normal labor
- c) For delivery of the head in breech presentation
- d) For delivery of the legs in breech presentation
- e) For delivery of the head in brow presentation

SUPPORT 41

Which of the following endometrial histopathological finding is suggestive for ectopic pregnancy?

- a) Secretory endometrium
- b) Chorionic villi
- c) Proliferative endometrium
- d) Decidual reaction
- e) Endometrial hyperplasia

The plane from the sacral promontory to the inner posterior surface of the pubis symphysis is an important dimension of the pelvis for normal delivery. What is the name of this plane?

- a) True conjugate
- b) Obstetric conjugate
- c) Diagonal conjugate
- d) Bi-ischial diameter
- e) Oblique diameter

A 27 year old lady, she has a 4 month old baby. Her menstrual cycles are heavy but regular. She has a family history of diabetes. On examination, her blood pressure 150-90, BMI 33 kg/m². Which of the following is the best contraceptive method for her?

- a) Copper intrauterine device
- b) Combined hormonal device
- c) Hormonal intrauterine device
- d) Barrier methods
- e) Implanon

SUPPORT 41

A 25 years old female visit her physician for delayed menstruation and symptoms of pregnancy in form of nausea and vomiting. Her LMP was 23-4-2023. She has history of PCO. You ordered serum pregnancy test and the result confirmed pregnancy.

Which of the following is true regard her EDD:

- a) EDD will be 23-12-2023
- b) EDD will be 30-1-2023
- c) EDD will be 1-2-2023
- d) EDD should be assessed by ultrasound examination
- e) EDD can be calculated later on by measuring fundal height

You are caring for female on the 7th day of the postpartum period after vaginal delivery.

Which of the following is a proper service to provide to this lady?

- a) Advise her to use vaginal douches to prevent infection
- b) Tetanus toxoid should be offered at this time
- c) Examine her for edema lower limb and chest pain
- d) Advice her to regain her usual physical activity after the end of puerperium
- e) Avoid iron and calcium supplementation during lactation

A pregnant lady in the 10th week of gestation. Which of the following is NOT included in the educational message for her?

- a) Nutritional counseling
- b) Personal hygiene and breast cancer
- c) Exercise and proper daily activity
- d) Labor signs and onset of labor
- e) Dangerous signs during pregnancy

SUPPORT 41

A 23 year old female with regular menstruation comes for the clinic seeking for IUD insertion. Which of the following is the optimal period for inserting an IUD?

- a) The IUD can be inserted at any time throughout the menstrual cycle
- b) On the last day of menstruation
- c) Few days before expected time of menstruation
- d) On the first day of menstruation
- e) About 4-6 day after cessation of menstruation

You are conducting premarital counseling for a couple few month before marriage.

Which of the following should be included in the counselling session?

- a) It is the best time to discuss different contraceptive methods that can be used in between pregnancies
- b) Genetic testing is an integral part of premarital care for any couple
- c) Counselling for chronic medical condition and referral when needed
- d) Developing good oral hygiene because dental interventions during pregnancy increase the incidence of abortions
- e) A screening for sexually transmitted diseases because they can be a causative agent in preterm labor and ectopic pregnancy

Body mass index BMI 24 kg/m². When she was seen in the clinic at 14 weeks of gestation, she had gained 1.8 kg since conception.

How would you interpret this?

- a) The woman weight gain is appropriate for this stage of pregnancy
- b) This weight gain indicates that the woman infant is at risk for intrauterine growth restriction
- c) This weight gain cannot be evaluated until the woman has been observed for several more weeks
- d) This weight gain indicates possible gestational hypertension
- e) According to her body mass index she should gain 3-4 kg at 12 weeks of gestation

SUPPORT 41

Which statement is most accurate regarding response to iron therapy in patient with iron deficiency anemia?

- a) Response to iron therapy can be documented by an increase in reticulocyte count 3 weeks after the initiation of iron therapy
- b) The benchmark for successful iron supplementation is a 4 g/dl increase in hemoglobin level in 3 weeks
- c) The benchmark for successful iron supplementation is a 2 g/dl increase in hemoglobin level in 5-10 days
- d) Response to iron therapy can be documented by an increase in reticulocytes 5-10 days after the initiation of iron therapy
- e) Increase in the ferritin level to the normal value within one week of therapy

SUPPORT 41

- A 27 year old female who have started using combined oral contraception (COC) two months ago, she has no relevant medical problem, her child aged 15 month and it is the first time to use a contraceptive method.

1. What are the possible causes for irregular bleeding with COC in this female?

2. List steps for managing this situation

3. List contraindication for COC

- A 36 year old pregnant woman (16 week gestation) is seen at her 2nd antenatal care visit. This is her fourth gestation. First child was delivered vaginally while the subsequent children were delivered by cesarean section. Her last child aged 11 months. She has a family history of neural tube defect. On examination temperature 37c HR 95, BP 125/80. Hb 9.3 g

1. List risk factor for high-risk pregnancy in this lady

2. Describe screening test that should be performed at this time

3. Describe supplementations required for this lady (including doses)

- A 32 year old multigravida was admitted to hospital in labor at full term. The patient had slow cervical dilatation progress in spite of strong contractions and rupture of membranes. Then suddenly the patient feels severe abdominal pain followed by cessation of contractions and vaginal bleeding. On examination, pulse was 130/min and weak, temperature 37c, BP 80/50. Uterus was retracted and felt separated from the fetus and FHR was absent. On vaginal examination the fetal head become higher with vaginal bleeding.

1. What is the most probable diagnosis

2. Mention the two types of this condition

3. List two maternal complication of this condition

4. Outline 2 surgical management options of this obstetric emergency condition

- A 28 years old multigravida about 7 weeks gestation was admitted to hospital suffering from lower abdominal pain and light brown vaginal bleeding. On examination, pulse was 90/min, temperature 37c, BP 120/70. On vaginal examination there was cervical movement tenderness (jumping sign). Urine pregnancy test was positive. No visible intrauterine sac by ultrasound.

1. What is the most probable diagnosis

2. How can you confirm diagnosis of this condition

3. Enumerate 4 patient selection criteria for medical treatment of this condition

4. Outline 2 surgical management options of this clinical condition

SUPPORT 41

- A 29 year old primigravida about 32 weeks gestation was admitted to hospital suffering from intermittent colicky lower abdominal pain and increased vaginal discharge. On examination, pulse was 80/min, temperature 37c, BP 110/70 mmHg. Active uterine contractions were recorded by CTG with 2 contractions every 10 minutes. On vaginal examination, cervical dilatation was 2 cm with 50% effacement.

1. What is the most probable diagnosis

2. Enumerate two methods to enhance fetal lung maturity in this condition.

3. Enumerate 2 commonly used drugs for cessation of uterine contractions in this condition.

- Extended Matching Question

Options:

- A. Complete abortion
- B. Incomplete abortion
- C. Threatened abortion
- D. Missed abortion
- E. Inevitable abortion
- F. Septic abortion
- G. Habitual abortion

Questions:

1. Uterine bleeding at 12 weeks gestation accompanied by cervical dilatation without passage of tissue. **E**

2. Passage of some but not all placental tissue through the cervix at 9 weeks gestation **B**

SUPPORT 41

A 23 years old G2 P1+0 previous one cesarean section is admitted to hospital with history of dribbling for 24 hours at a period of amenorrhea of 32 weeks. Speculum examination confirms dribbling. The liquor is offensive. Her temperature is 38c. High reactive protein. No uterine contraction. FHR is 170 per minute. What is the most appropriate treatment?

- a) Administer broad spectrum antibiotics and corticosteroids and manage conservatively if the temperature decreases
- b) Perform cesarean section immediately after administering 1 dose of corticosteroids and broad spectrum antibiotics
- c) Augment labor with intravenous infusion of oxytocin
- d) Administer tocolytics therapy
- e) Repeat C-reactive protein 24 hours after commencing broad spectrum antibiotics

The placenta of twins can be:

- a) Dichorionic and monoamniotic in dizygotic twins
- b) Dichorionic and monoamniotic in monozygotic twins
- c) Monochorionic and monoamniotic in dizygotic twins
- d) Monochorionic and diamniotic in monozygotic twins
- e) Dichorionic and diamniotic in monozygotic twins

All of the following are maternal changes during pregnancy EXCEPT:

- a) Increase cardiac output
- b) Increase alveolar residual volume
- c) Increase fibrinogen
- d) Increase glomerular filtration rate
- e) Increase stroke volume

SUPPORT 41

Abnormal uterine action includes all of the following EXCEPT:

- a) Constriction ring
- b) Cervical dystocia
- c) Colicky uterus
- d) Pathological retraction ring
- e) Cervical insufficiency

Family

Which statement is most accurate regarding the management of iron deficiency anemia in pregnant woman?

- a) Daily oral iron with 30 mg to 60 mg of elemental iron is recommended for pregnant women with hemoglobin level 10.5 g
- b) The response to iron therapy would be an increase in serum ferritin into the normal range within 5-10 days
- c) The response is different with oral or intravenous administration
- d) Parenteral iron therapy is indicated in pregnant women beyond the first trimester who cannot tolerate oral iron
- e) Absorption of iron may be improved by drinking coffee at the time the iron supplement is taken

A 29-year-old obese woman with type 2 diabetes mellitus is asking you about progestin-only pills as a method of contraception. Which of the following is true?

- a) Progestin-only pills are highly contraindicated in women with diabetes
- b) Progestin-only pills would increase her risk of thromboembolic events
- c) Progestin-only pills increase the risk for pelvic inflammatory disease
- d) Progestin-only pills isn't preferred for lactating women
- e) Progestin-only pills should be taken every day of the month, without a hormone-free period

SUPPORT 41

You are counselling a 26 years old female about contraceptive methods. Which of the following is true as regard progesterone releasing vaginal rings?

- a) The ring is inserted in the vagina and left in place for up to 90 days
- b) The ring is not suitable for lactating females
- c) It is contraindicated in obese females
- d) Six rings can be used, one after another, for approximately one year postpartum
- e) Start each new ring one week after removal of the previous ring

- List five contraindications for intrauterine device insertion
- A 33 year old pregnant lady 20 weeks gestation came for her antenatal visit. She has an appointment for ultrasound testing. She has no chronic health problem. Her BMI before pregnancy was 25.9 kg/m². She is worried about the possibility of having gestational diabetes.

1. What is the proper total body weight gain during pregnancy for this female?
2. List components for proper nutritional advice during pregnancy
3. Describe screening for gestational diabetes
4. Enumerate 4 contraindications to the previously used drugs

SUPPORT 41

- A 34 years-old G2 with previous one cesarean section. She was about 34 weeks gestation and admitted to hospital suffering from painless recurrent mild attacks of bright red vaginal bleeding. On examination, pulse was 95/min, temperature 37°C, BP 100/70. On abdominal examination, there was a relaxed uterus and FHR 150/min. By ultrasound, the placenta was completely covering the cervix.

1. What is your provisional diagnosis of this condition.
2. Enumerate 2 common factors that predispose to this condition.
3. How can you manage this condition according to the amount of vaginal bleeding and gestational age.
4. Enumerate 2 indications of conservative management of this condition.

Final 38

- Enumerate 2 main investigations for vesicular mole, list 2 surgical management options, and mention 6 indications of chemotherapy in post molar period.
- Define Hyperemesis Gravidarum, enumerate 4 investigations, and outline 4 steps of management.
- Enumerate steps of management of shoulder dystocia.
- A 32 years-old multigravida was admitted to hospital in labor at full term. Examination revealed: cephalic presentation, cervix dilated 4 cm, membranes ruptured and umbilical cord is felt in the vagina. A fetal heart rate is auscultated at 100 beats/minute.
 1. List four clinical conditions that predispose to this critical situation.
 2. Mention the cause of fetal distress in this condition.
- A 28 years-old multigravida about 32 weeks gestation was admitted to hospital suffering from sudden, large volume of fluid leakage without uterine contractions. On examination, pulse was 80/min, temperature 37°C, BP 120/70. Uterus was lax, FHR was 134/min. On Cusco speculum examination, there was a pool of fluid in the posterior vaginal fornix.
 1. What is the most probable diagnosis?
 2. What is the most serious complication of this condition?
 3. Enumerate 2 investigations of this condition.
 4. Outline the management of this clinical scenario.

SUPPORT 41

- You are asked to see a woman in the antenatal clinic. She is 37 years old and pregnant with her third child. Her previous children were both born by vaginal delivery after induction of labor for postdates. At her last appointment at 36 weeks' gestation, the midwife suspected that the baby was in a breech presentation. On examination, BP 130/80 and abdominal examination suggests a breech presentation. U/S report confirmed breech presentation, with fetal weight 3.2 KG, placenta high anterior with normal volume of amniotic fluid.

1. What is the most common fetal cause of breech presentation?

2. Enumerate 2 maternal complications of breech delivery.

3. Outline the 3 management options available for this woman.

4. Enumerate the 3 techniques for delivery of the after coming head.

- A 39-year-old woman in her first pregnancy delivered twin sons vaginally 2 h ago, there were no significant complications. The midwife recorded both placenta as appearing complete. The lochia has been heavy since delivery, but the woman is now bleeding very heavily and passing large clots of blood. The woman is conscious but drowsy and pale. The temperature is 37°C, blood pressure 120/70 mmHg and heart rate 112/min. The uterus is palpable to the umbilicus and feels soft. On vaginal inspection there is a second-degree tear which has been sutured but are unable to assess further due to the presence of profuse bleeding.

1. What is your provisional diagnosis of this condition?

2. Enumerate the 4 common causes of this condition.

3. Enumerate the sequence of management options in this condition.

SUPPORT 41

- Extended Matching Questions

Options:

- a) Fenestrated placenta
- b) Succenturiate placenta
- c) Vasa previa
- d) Placenta previa
- e) Membranous placenta
- f) Placenta accreta

Questions:

1. A 33-year-old G2P1 is undergoing an elective repeat cesarean section at term. The infant is delivered without any difficulties, but the placenta cannot be removed easily because a clear plane between the placenta and uterine wall cannot be identified. The placenta is removed in pieces. This is followed by uterine atony and hemorrhage. **F**
2. A 22-year-old G3P2 undergoes a normal spontaneous vaginal delivery without complications. The placenta is spontaneously delivered and appears intact. The patient is later transferred to the postpartum floor where she starts to bleed profusely. Physical examination reveals a boggy uterus and a bedside sonogram indicates the presence of placental tissue. **B**
3. A 34-year-old G6P5 presents to labor and delivery by ambulance at 33 weeks gestational age complaining of the sudden onset of profuse vaginal bleeding. The patient denies any abdominal pain or uterine contractions. She denies any problems with her pregnancy to date but has had no prenatal care. The fetal heart rate tracing is normal. There are no contractions on the tocometer. **D**

Options:

- a) Complete abortion
- b) Incomplete abortion
- c) Threatened abortion
- d) Missed abortion
- e) Inevitable abortion

Questions:

1. Uterine bleeding at 12 weeks gestation accompanied by cervical dilation without passage of tissue. **E**
2. Passage of some but not all placental tissue through the cervix at 9 weeks gestation. **B**
3. Fetal death at 15 weeks gestation without expulsion of any fetal or maternal tissue for at least 8 weeks. **D**
4. Expulsion of all fetal and placental tissue from the uterine cavity at 10 weeks gestation. **A**

SUPPORT 41

A patient of 36 weeks gestation presents with hypertension, blurring of vision and headache. Her blood pressure reading was persistent 180/120. Proteinuria +3. How will you manage this patient?

- a) Admit the patient and observe
- b) Admit the patient, start antihypertensive and continue pregnancy
- c) Admit the patient, start antihypertensive, MgSO₄, and terminate the pregnancy
- d) Start antihypertensive and follow up in outpatient department
- e) IV furosemide and follow up of blood pressure

A 24-year-old G1P0 presents at 6 weeks with a BhCG 16,000 mIU/mL and mild left lower quadrant pain. Transvaginal US showed unruptured 3-cm ectopic pregnancy without fetal cardiac motion. Which of the following is the best management option?

- a) Methotrexate 30 mg/m² IM
- b) Methotrexate 50 mg/m² IM
- c) Salpingectomy
- d) Laparoscopic salpingostomy
- e) Repeat BhCG in 48 hours

A 30 years old female G2P1+0 at 38 weeks gestation presents with labor pains. On examination, cervix is 5 cm dilated and the fetus was found to be in face presentation with left mento-anterior position. The next step of management is:

- a) Cesarean section
- b) Forceps
- c) Ventouse delivery
- d) Expectant management
- e) Jaw flexion–shoulder traction

SUPPORT 41

A 28 years eclamptic woman develops convulsions. The first measure to be done is:

- a) Care of airway
- b) Immediate delivery
- c) Give magnesium sulphate
- d) Sedation of the patient
- e) Give antihypertensive treatment

The plane from the sacral promontory to the inner posterior surface of the pubic symphysis is an important dimension of the pelvis for normal delivery. What is the name of this plane?

- a) True conjugate
- b) Obstetric conjugate
- c) Diagonal conjugate
- d) Bi-ischial diameter
- e) Oblique diameter

Which of the following is the most accurate for dating the pregnancy?

- a) Determination of the uterine size on pelvic examination
- b) Quantitative serum HCG level
- c) Crown–rump length on abdominal and vaginal ultrasound
- d) Determination of progesterone level
- e) Determination of estrogen level

During pregnancy the blood volume increases by 40%. The increase in blood volume in normal pregnancy is made up of which of the following?

- a) Erythrocytes
- b) More erythrocytes than plasma
- c) Plasma only
- d) Neither plasma nor erythrocytes
- e) More plasma than erythrocytes

SUPPORT 41

A patient is seen in the early third trimester of pregnancy with acute onset of chills and fever, nausea, and backache. Her temperature is 39°C. The urine sediment reveals many bacteria and WBCs. Which of the following is the likely diagnosis?

- a) Acute appendicitis
- b) Ruptured uterus
- c) Pyelonephritis
- d) Abruptio placenta

Most common congenital malformation seen in diabetic pregnant mother:

- a) Liver defect
- b) Cardiac defect
- c) GIT defect
- d) Lung defect
- e) Skeletal defect

A 25 years old female, Gravida 5, Para 0+3, with history of recurrent early pregnancy loss. Which of the following investigations is NOT ordered?

- a) Thyroid function test
- b) Genetic testing and karyotyping
- c) Testing for TORCH
- d) Testing for antiphospholipid syndrome
- e) 3-D pelvic ultrasound for uterine anomalies

A 33 year-old pregnant lady with gestational age of 38 weeks. CTG monitoring revealed transient decrease in fetal heart rate after beginning of uterine contractions and subsided after ending of the contractions. What is your diagnosis?

- a) Early deceleration
- b) Late deceleration
- c) Variable deceleration
- d) Sustained deceleration
- e) Bradycardia

SUPPORT 41

In which presentation the fetal head is deflexed?

- a) Occipito-anterior position
- b) Occipito-posterior position
- c) Face presentation
- d) Brow presentation
- e) Compound presentation

A patient is experiencing an arrest of descent. During the evaluation one can feel that it is a vertex presentation with the sagittal suture transverse or oblique but closer to the symphysis than the promontory. What is this specific condition called?

- a) Posterior asynclitism
- b) Internal rotation
- c) Extension
- d) Anterior asynclitism
- e) Restitution

In preterm labor, the risk of fetal complication is highest with the use of the following drug:

- a) Indomethacin
- b) Nifedipine
- c) Ritodrine
- d) Magnesium sulphate
- e) Exupine

What is the correct statement in twin-to-twin transfusion syndrome?

- a) Weight of both babies is less than 2 kg
- b) Due to placental vascular communications
- c) Babies appear plethoric
- d) The first baby is breech and the second is cephalic
- e) Occurs commonly in conjoined twins

SUPPORT 41

At delivery, a perineal laceration tore through the skin of the fourchette, vaginal mucous membrane, and the fascia and perineal muscles of the perineal body but NOT the anal sphincter or mucosa. This should be recorded in the medical record as what type of laceration?

- a) First-degree
- b) Second-degree
- c) Third-degree
- d) Fourth-degree
- e) Complete

Ventouse in second stage of labor is contraindicated in:

- a) Persistent occipito posterior position
- b) Heart disease
- c) Uterine inertia
- d) Maternal heart disease
- e) Preterm labor

All of the following are possible causes of disseminated intravascular coagulopathy EXCEPT:

- a) Abruptio placenta
- b) Amniotic fluid embolism
- c) Intrauterine fetal death
- d) Diabetes mellitus
- e) Eclampsia

What dose of glucose has been recommended by WHO for oral glucose tolerance test during pregnancy?

- a) 75 gm
- b) 50 gm
- c) 150 gm
- d) 200 gm
- e) 100 gm

SUPPORT 41

True statement about symmetrical IUGR in comparison with asymmetrical IUGR:

- a) Worse prognosis
- b) Brain tissue sparing
- c) Head larger than abdomen
- d) More common
- e) Due to chronic malnutrition and placental insufficiency

A 35-year-old pregnant female who has two children. She plans to get pregnant again. She asked her physician if she has to repeat the genetic testing before pregnancy. Which of the following is NOT an indication for genetic testing in this female?

- a) There is history of recurrent abortion
- b) Consanguinity between parents
- c) Congenital abnormalities in the offspring
- d) Previous history of stillbirth
- e) History of previous child with thalassemia

A 42-year-old pregnant woman was very worried during ANC visit that she has missed the right time to have her screening test for Down's syndrome. She is now 17 weeks pregnant. You counsel her about the quadruple test. Which of the following is true?

- a) The best time for quad test is from 12–16 weeks of gestation
- b) It includes unconjugated estriol, HCG, alkaline phosphatase and inhibin A
- c) It includes beta HCG, a-fetoprotein, nuchal translucency and inhibin A
- d) It includes unconjugated estriol, a-fetoprotein, inhibin A, beta HCG
- e) It can be done in third trimester with 20% false negative results

SUPPORT 41

You started a 20-year-old woman on COCs 2 months ago. She returns to your office asking to discontinue their use because of side effects. Which of the following side effect of COCs is the most frequent reason for discontinuing their use?

- a) Irregular bleeding
- b) Breast tenderness
- c) Fluid retention
- d) Headache
- e) Nausea

As regard nutritional counseling, which of the following is true regarding folic acid supplementation?

- a) The recommended dose of folic acid is 0.2 mg once per day beginning at least one month prior to conception
- b) Folic acid supplementation has a role in preventing trisomy
- c) Measurement of maternal folate levels before supplementation is recommended
- d) Diets rich in animal proteins, fresh leafy vegetables, and legumes would be sufficient for pregnant lady
- e) Dose of folic acid is 4 mg if there was a previous history of neural tube defect

SUPPORT 41

- Describe components of premarital counselling.
- A 42 years old woman visits the clinic complaining that she has forgotten to take the combined contraceptive pills (COC) for the previous three days because she was travelling on a vacation and she forgot to take her pill package. There are 12 pills left in the package. She has been on COC for 7 years. She has two children; the youngest is 8 years old. She has no medical problems, receives no medications, and she has family history of breast cancer. By examination, BP is 145/95, pulse is 78 b/m, respiratory rate is 20 c/m. Body weight is 75 kg, height is 155 cm. Fasting blood glucose is 125 mg/dL.

1. What is the proper management for this situation?
2. Why would you advise her to consider changing her contraceptive method?
3. What would be the suitable method for her?
4. List advantages of the chosen method.

Final 37

- List the causes of preterm premature rupture of membranes, investigations to confirm the diagnosis, and outline the management plan.
- Causes of shoulder dystocia and outline the management plan and maneuvers for successful delivery.
- Enumerate five causes of pathological neonatal jaundice.
- List the causes of first trimester bleeding.
- In her fourth pregnancy, a woman aged 26 is currently attending the obstetrics clinic at 20 weeks gestation. Her obstetric history includes a spontaneous vaginal delivery four years ago succeeded by two miscarriages at 8 weeks and 14 weeks, respectively, without an identified cause. Throughout the current pregnancy, she has maintained good overall health. She is confirmed to be Rhesus negative, and the results of her booking and subsequent hematological blood tests are as follows: Blood group O negative, Anti-D antibodies present at a titer of 4 IU/mL at 10 weeks, 4 IU/mL at 16 weeks, and 9 IU/mL at 20 weeks.

1. What is the most likely diagnosis?

2. What are the potential consequences of this problem?

3. Outline the management for this patient.

SUPPORT 41

- At 32 weeks and 4 days' gestation in her first pregnancy, a 23-year-old woman expresses concern about reduced fetal movements. Normally experiencing more than 10 movements daily, she noticed only two yesterday and none today. She initiated prenatal care at 10 weeks with all her initial blood tests returning normal results, except for the discovery that she was not immune to rubella, prompting postnatal vaccination plans. Her 11–14-week scan, nuchal translucency test, and anomaly scan were all within normal ranges. During examination, her blood pressure measures 137/73 mmHg, and her pulse is 93/min. She shows no signs of fever. The symphysis-fundal height of the uterus is 31 cm, and fetal breech presentation is noted on examination. When using a hand-held Doppler, no fetal heartbeat is auscultated. Consequently, an immediate ultrasound scan is arranged, confirming the diagnosis of intrauterine fetal death.

1. What is the best management for this patient?

2. List 6 maternal or fetal causes that may lead to IUFD.

3. Enumerate 3 specific investigations that should be performed to establish a possible cause.

- A 20-year-old woman is experiencing her first unplanned pregnancy. Diagnosed with type 1 diabetes at the age of 15, she has been managing her condition with both long-acting and short-acting insulin. Two weeks ago, she received a positive pregnancy test, and according to her estimated dates, she is currently at 7 weeks and 5 days of gestation. Apart from her diabetes history, there are no other notable gynecological or medical concerns. Her HbA1c level is recorded at 7.5%, and urinalysis indicates a presence of ++ glucose.

1. Create a management plan for this patient.

2. Summarize the impacts of DM on both mother and fetus.

SUPPORT 41

The following procedure allows the earliest retrieval of DNA for prenatal diagnosis:

- a) Fetoscopy
- b) Chorionic villous sampling
- c) Amniocentesis
- d) Percutaneous umbilical blood sampling

Contraindication to medical therapy (Methotrexate) in tubal pregnancy is:

- a) Size of the ectopic gestational sac is 3 cm or less
- b) History of active hepatic or renal disease
- c) No fetal heart motion on ultrasound
- d) Absence of active bleeding

Which of the following is the first sign of magnesium sulphate toxicity?

- a) Respiratory depression
- b) Cardiac depression
- c) Loss of deep tendon reflexes
- d) Seizures

Which of the following tocolytics is associated with reversible oligohydramnios?

- a) Indomethacin
- b) Magnesium sulphate
- c) Ritodrine
- d) Terbutaline

Detection of which of the following in cervicovaginal secretions is a powerful clinical predictor of subsequent preterm birth?

- a) Decidual relaxin
- b) Fetal fibronectin
- c) Interleukin-1
- d) Tumor necrosis factor

SUPPORT 41

Which of the following ultrasound measurements is the most reliable index of fetal weight?

- a) Biparietal diameter
- b) Abdominal circumference
- c) Femur length
- d) Intrathoracic ratio

Which sonographic measurement in a growth-restricted fetus correlates best with significant perinatal mortality?

- a) Biparietal diameter
- b) Abdominal circumference
- c) Femur length
- d) Oligohydramnios

Indication of elective C.S. in twin pregnancy include all EXCEPT:

- a) Weight less than 2 kg
- b) First twin vertex
- c) Previous myomectomy
- d) Conjoined twins

Complete uterine rupture is characterized by the following EXCEPT:

- a) Massive hemorrhage
- b) All layers of the uterus are involved
- c) The visceral peritoneum may be intact
- d) The fetus is almost always dead

The best course of action for a complex 12-cm adnexal mass noted at 18 weeks gestation is:

- a) Review after 4 weeks
- b) Immediate laparotomy
- c) Sonographically directed aspiration
- d) Laparoscopy and aspiration

SUPPORT 41

The mechanism by which betamethasone reduces respiratory distress syndrome is by:

- a) Increasing cytokine production
- b) Increasing prostaglandin production
- c) Increasing surfactant production (phosphatidylglycerol production)
- d) Increasing alveolar growth

The succenturiate placenta consists of:

- a) Two equal placental lobes connected by placental tissue
- b) Small accessory lobe connected to the main placenta by placental tissue
- c) Small accessory lobe connected to the main placenta by membranes
- d) Small accessory lobe not connected to the main placenta

Regarding estrogens in pregnancy:

- a) Estradiol is the main estrogen during pregnancy
- b) They are decreased in cases of hydrocephalus
- c) They are secreted principally by the cytotrophoblasts
- d) The fetus contributes to production
- e) They stimulate the alveolar development in breasts

The part of the uterus that becomes the lower uterine segment is:

- a) Cervix
- b) Isthmus
- c) Corpus
- d) Interstitium

The following statement regarding blood composition in normal pregnancy is CORRECT:

- a) The packed cell volume rises
- b) There is a rise in the iron-binding capacity
- c) The blood cholesterol decreases
- d) The total red cell mass falls about 20%

SUPPORT 41

During pregnancy, the renal glomerular filtration rate (GFR) can increase by as much as:

- a) 25 percent
- b) 50 percent
- c) 75 percent
- d) 100 percent

وَأَخِرُ دَعْوَاهُمْ أَنِ الْحَمْدُ لِلَّهِ رَبِّ الْعَالَمِينَ